جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ارزاق ابراهيم احمد عبدالجواد
المحافظة : الجيزه **الحي / المدينة :** البدرشين **العنوان :** الجيزه
رقم البطاقة : 27610202102802 **رقم التليفون :** 01025858996 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 20-10-1976 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنه و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ارزاق ابراهيم احمد عبدالجواد
National ID: 27610202102802

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Contrast-enhanced CT of chest, abdomen and pelvis.**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



٠١٠٣٣٢٩٤٥٧
١٤٤٦٧٦٤٩٣٥



National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

07/26/2026

MDT FORM

Name:	ارزاق ابراهيم احمد عبدالجواد	**MRN:**	11876
Age:	50 Y	**ID:**	27610202102802
Menopausal Sta	POST	**Phone:***	01003239457
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	-VE
Gender			
CO Morbidity:	MED FREE	**Reason Of MDT:**	POST OP (OUTSIDE THE HOSPITAL)
Surgical History:*	TONSILLECTOMY RT EAR TEMPANOPLASTY	**ECOG Performance Status**	○ 0 ◉ 1 ○ 2 ○ 3 ○ 4

Clinical Data:

Name: ارزاق ابراهيم احمد عبدالجواد	
MR. : 11876	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 49-year-old individual who is postmenopausal. The clinical history is notable for a right-sided breast cancer previously treated with right breast-conserving surgery on 24/5/2026 performed outside Gustave. The initial pathology was invasive ductal carcinoma, grade 3, with medullary features, staged as pT1 N3 (10/15 nodes), triple-negative, Ki-67 25 %. A pre-operative PET-CT demonstrated right supraclavicular lymph node uptake with SUV 10 and a 1.8 cm lesion. Post-operative MRI of the right breast showed a BIRADS 4A non-mass-like enhancement. The patient was recommended to receive two weeks of taxol and carboplatin.

A true-cut biopsy of the right breast lesion performed on 17/8/2026, corresponding to the post-operative MRI BIRADS 4 non-mass enhancement, revealed changes consistent with resection site alterations without residual disease. The biomarker profile confirmed triple-negative status with Ki-67 25 %. Imaging findings include the aforementioned post-operative MRI BIRADS 4A non-mass enhancement and the prior PET-CT supraclavicular node involvement; no additional metastatic disease was reported.

The referring team recommended two weeks of taxol and carboplatin. The multidisciplinary team's outcome and plan is to continue systemic therapy and proceed with completion mastectomy after the current treatment.

تحريرا في ,

التاريخ :	**23-09-2026**	**الوقت :**	

الطبيب المعالج :

يعتمد ,

BX: IDC G3 NST WITH MEDULLARY LIKE FEATURES

ER: 0/8 **PR:** 0/8 **HER2:** 0 **KI67:** 25

History:

RT BREAST MASS , THEN RT WLE +AC OUTSIDE THE HOSPITAL WITHOUT BIOPSY

Sonomammography:

***THE INITIAL SM :

RT UOQ IRREGULAR ISODENSE LESION WITH CALCIFIC FOCI OVERLYING THE LESION . LESION LOCATED 20MM DEEP TO SKIN IN ZONE C AT 9 O'CLOCK 10*5*11.7 MM .MULTIPLE ENLARGED MORE THAN 3 PATHOLOGICAL LN

***POST OP MRI:

POST BCS+AC , POST OP CHANGES WITH UOQ FOCAL AREA OF NON MASS ENHANCEMENT BIRAD 4A 3*1 CM

***POST OP PET CT:

METABOLICALLY ACTIVE RT AXILLARY LEVEL 2 AND SUPRACLAVICULAR LN

Biomarker/Molecular

***PATHOLOGY REVISION:

FREE RESECTION MARGINS

AX. LN 10/15

ABSENT DCIS, ABSENT MICROCALCIFICATION

ABSENT PERINEURAL INVASION

ANGIOINVASION PRESENT

TUMOR SIZE 1*0.5CM

Mets Workup:

Recommendations:

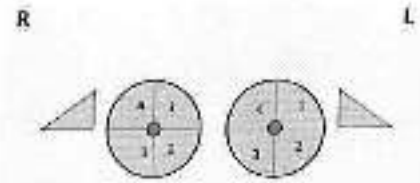
MDT Outcome & Plan

1- for trial of clip bracketing of non mass area in ucq if rejected by intervention radio so for completion mastectomy post ttt

2- Portcath insertion - BRCA testing

3- Start Paclitaxel - Carboplatin x 12 weeks -> then 4 AC

+/- pembrolizumab if approved



T N M

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

GUSTAVE ROUSSY
Gustave Roussy International Egypt Pathology Lab
جوستاف روسي

Patient Name	ارزاق ابراهيم احمد	Case No.	340122
Sex/Age	Female 49 Y	Patient No.	24416
Registered Date	04/7/2026 03:44:50 PM	Referred By Dr.	Out Patient
Validation Date	08/7/2026 11:35:06 AM	Branch	Gustave Roussy
Reporting Date	08/7/2026 11:46:52 AM	Pathology No.	SR-778/26

Pathology report
(slide revision)

Clinical Data: Right breast cancer, conservative surgery and Axillary Lymph node dissection were done outside GRI-e

Nature of Specimen: Slide revision

Gross Examination:
-Eight FFPE blocks labelled (202296 and 200595-26) and 17 slides were received for slide revision

Microscopic examination:
- **Histologic type:** Invasive breast carcinoma, NST with medullary like features
- **Histologic grade:** 3 (differentiation 3, pleomorphism 3, mitosis 2)
- **DCIS:** Absent
- **Microcalcification:** Absent
- **Angioinvasion:** Present
- **Perineural invasion:** Absent
- **Resection margins:** Refer to previous report
- **Tumor size:** 1x0.5 cm (as described in previous report)
- **Axillary Lymph nodes:** Positive for tumor deposits in 10/15 Lns with extranodal extension
- **Stage:** pT1b N3

Review of IHC slides
* IPT (All controls showed appropriate reactivity):
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67%: 25%.

Conclusion:
Right breast cancer, Brest conservative surgery and axillary lymph node dissection (slide revision)
- **Invasive breast carcinoma, NST with medullary-like features**
- **Resection margins: Free (as described in previous report)**
- **Axillary Lymph nodes: Positive for tumor deposits in 10/15 LNs with extranodal extension**

CAP
غرفة رقم (4) - الدور الأول - عمارة رقم (387) - شارع كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 387- Nile Corniche - Misr El Qadema - Cairo, Egypt

Breast Biopsy
[[DOCTYPE:BREAST_BIOPSY]]

GUSTAVE ROUSSY

Patient Name	[illegible]	Case No.	340122
Sex/Age	Female 49 Y	Patient No.	24416
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Validation Date	08/7/2026 11:35:06 AM	Branch	Gustave Roussy
Reporting Date	08/7/2026 11:46:52 AM	Pathology No.	SR-778/26

IPT (slide revision)
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- KI67%: 25%.

***Scoring system: "ER, PR"*
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
*** Scoring system: "Her2"*
Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Prof. Dr. Maha Guimei

Breast Biopsy
[[DOCTYPE:BREAST_BIOPSY]]

Patient Name	ارزاق ابراهيم احمد	**Case No.**	**358678**
Sex/Age	**Female 49 Y**	**Patient No.**	**25988**
Registered Date	**17/8/2026 02:05:19 PM**	**Referred By Dr.**	**Interventional Radiology**
Validation Date	**05/9/2026 11:00:47 AM**	**Branch**	**Gustave Roussy**
Reporting Date	**05/9/2026 11:20:56 AM**	**Pathology No.**	**2607/26**

Pathology Report

Clinical Data: Right breast UOQ post-operative sequelae in a status post right breast conservative surgery for pathologically proven carcinoma.

Nature of Specimen: Tru-cut biopsy, collected on 17/08/2026, Cold ischemia time: 20 seconds.

Gross Examination:
Three grey, white tissue cores, ranging in length between 0.3 cm & 0.8 cm, totally submitted.

Microscopic Examination:
- Histopathological examination reveals cores of tissue showing fat necrosis & foreign body multinucleated giant cell reaction. No evidence of malignancy in section examined.

Diagnosis:
Right breast lesion (post-right breast conservative surgery status), Tru-cut biopsy:
- Consistent with resection site changes (fat necrosis & foreign body multinucleated giant cell reaction).
- No malignancy.

Signature
Prof. Dr. Gamal Abd El-Atty.

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient's Name:	**Arzaq Ibrahim Ahmed Abdel Jawad**
Date:	**2026-08-22**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Neck Ultrasonography

multiple enlarged matted right supracalvicualr L.Ns seen showing irregular shape largest measuring 3cm with hypoechoic centers

The right and left lobes and isthmus of thyroid gland are of normal size, homogenous echo pattern, with regular outline.

Normal vascularity at CCDI.

No cystic or solid nodules detected.

The trachea is centralized.

Both carotid sheaths are free.

Conclusions

Right supraclavicualr highly suspicious pathological L.Ns as described for further assessment

Dr.Sahl Mahmoud

Ultrasound
[[DOCTYPE:ULTRASOUND]]

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Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	ارزاق ابراهيم احمد	**Case No.**	**358678**
Sex/Age	**Female 49 Y**	**Patient No.**	**25988**
Registered Date	**17/8/2026 02:05:19 PM**	**Referred By Dr.**	**Interventional Radiology**
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Signature
Prof. Dr. Gamal Abd El-Atty.

غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name: ارزاق ابراهيم احمد عبدالجواد
MR. : 11876

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من | بشأن تقرير طبي للمريض

بقسم: | رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالتة

تاريخ وساعة الدخول : yyyy-mm-dd --:-- -- | تاريخ وساعة الخروج : yyyy-mm-dd --:-- --

وبعد الكشف والفحوصات والمتابعة وجد انة

wt 72 ht 165 sa 1.8 postmenop rt breast cancer undergone rt bcs on 24/5/2026 upfront outside gustave pt1 n3 10/15 idc with medullary feautures grade 3 triple -ve ki 25% initial pet ct rt supraclav ln suv 10 m 1.8 cm postop ,mri breast rt uoq birads 4 a non mass like enhancement

for meoadj keytruda

تحريرا في ,

الوقت : --:-- -- | التاريخ : yyyy-mm-dd

الطبيب المعالج : .....

يعتمد ,

alfa
L A B S

Collection Date: 02/08/2026
Review Date: 29/08/2026

Referring Doctor: أ.د خالد عبدالعزيز

Patient Name: الاستاذة/ ارزاق ابراهيم احمد
Patient No.: 203-25136
Accession No.: 203260823935
Age: 49Y

NEXT GENERATION SEQUENCING (NGS)

BRCA Report

Clinical information

Arzak Ibraheem is a Cancer patient, tested for germline mutations in BRCA 1,2 for the possible applicability for targeted therapy.

Test information

Extracted DNA using QIAamp DNA Mini kit from the patient's blood sample had successfully passed appropriate libraries according to the AmoyDx BRCA Pro Panel and sequenced on the MiSeq platform (Illumina). The minAmp (minimum depth) was not less than X200.

- The coding regions of BRCA1& BRCA2 genes have been investigated using NGS technology.
- BRCA1 & BRCA2 genes have been implicated in hereditary breast, ovarian, prostate and pancreatic cancer predisposition syndrome.
- For patients with BRCA1/2 mutations (actionable pathogenic or Likely pathogenic Variants), The PARP inhibitors show clear benefit (Gradishar et al., 2021)
- All identified variants are evaluated with respect to their pathogenicity and causality, and were categorized into classes (1 – 5) as recommended by ACMG guidelines, where Class (1) Pathogenic, Class (2) Likely pathogenic, Class (3) Variant of Uncertain Significance (VUS), Class (4) Likely benign, Class (5) Benign (Richards S, et al., 2015).
-Our variant classification pipeline included variant interpreter by Illumina, Clinvar and Varsome (Kopanos C. et al.,2019).
-Only pathogenic and likely pathogenic variants with actionable therapeutic application are reported.
-The test results were reviewed, interpreted and reported by our scientific and medical experts.
-The classification of variants can change over the time.

Test result

No Actionable Pathogenic or Likely Pathogenic Variants Detected in BRCA1,2 genes.

No large rearrangement or Copy Number Variations (CNV) in BRCA1,2 genes.

Interpretation

-The patient is not harboring Pathogenic or Likely Pathogenic Variants Detected in BRCA1,2 genes.

- The patient is unlikely to benefit of PARPI

Recommendations

MLPA analysis of BRCA1 and BRCA2 genes is recommended to rule out the presence of large deletions/duplications or copy number variations.

We recommend Follow up with the referring Physician.

Please feel free to contact NGS team at Alfa laboratories (NGS@alfalabs.com).

Pt. Branch: السيده زينب

Printed by: احمد حسن محمود4396

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معامل الفا
16191

alfa
L A B S

Collection Date: 02/08/2026
Review Date: 29/08/2026

Referring Doctor: أ.د خالد عبدالعزيز

Patient Name: الاستاذة/ ارزاق ابراهيم احمد
Patient No.: 203-25136
Accession No.: 203260823935
Age: 49Y

NEXT GENERATION SEQUENCING (NGS)

BRCA Report

Test limitations

1. Intronic variants/variants in untranslated region, promoters and intronic variants are not assessed using this method.

2. Variants not detected by the assay that was performed may impact the phenotype.

3. This test is using reference databases obtained from occidental communities.

4. Cases where the genetic variant is not identified may be due to limitations in current medical knowledge or testing technology.

5. Genetic tests are largely dependent on the clinical history, medical findings and family pedigree, and therefore clinical accurate interpretation of test results may require knowing the true biological relationships in a family. Misinterpretation of results may occur if the information provided is inaccurate or incomplete.

6. The interpretation of test results is done in accordance with the clinical history, family history and other medical and laboratory data.

7. Inaccurate results in genetic testing are very rare as these tests are highly specific and accurate.

References:

1.Garrett etal.,: BRCA1/BRCA2 :CanVIG-UK Gene-Specific Guidance 2026 (https://www.cangene-canvaruk.org/canvig-uk-guidance).2. Gradishar WJ, Moran MS, Abraham J, et al. NCCN Guidelines® Insights: Breast Cancer, Version 4.2021. J Natl Compr Canc Netw. 2021;19(5):484-493. Published 2021 May 1. doi:10.6004/jnccn.2021.0023
3. Kopanos C, Tsiolkas V, Kouris A, et al. VarSome: the human genomic variant search engine. Bioinformatics. 2019;35 (11):1978-1980. doi:10.1093/bioinformatics/bty897
4. Richards S, Aziz N, Bale S, et al. Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology. Genet Med. 2015;17(5):405-424. doi:10.1038/gim.2015.30

Authorized and Reviewed by:

Dr. Samar Saad
Consultant, MD

Prof. Sahar AbdlAtty
Head of the NGS Unit

Reviewed By:

Pt. Branch: السيده زينب

Printed by: 4396احمد حسن محمود

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معامل الفا
16191

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

09/08/2026

MDT FORM

Name:	ارزاق ابراهيم احمد عبدالجواد	**MRN:**	11876
Age:	50 Y	**ID:**	27610202102802
Menopausal Sta	postm	**Phone:***	01003239457
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:		**Reason Of MDT:**	postop mri birads 4 non mass enhancement initial decision for clipping but biops
Surgical History:*	bcs	**ECOG Performance Status**	○ 0 ◉ 1 ○ 2 ○ 3 ○ 4

Clinical Data:

BX:

ER: **PR:** **HER2:** **KI67:**

History:

postmenop rt breast cancer undergone rt bcs on 24/5/2026 upfront outside gustave pt1 n3 10/15 idc with medullary feautures grade 3 triple -ve ki 25% initial pet ct rt supraclav ln suv 10 m 1.8 cm

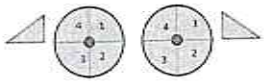
postop ,mri breast rt uoq birads 4 a non mass like enhancement

pt rec 2 weeks taxol carbopl

initial decision for clipping of this birads4 lesionfor wider excision after ending pseudoneoadj tt

true cut biopsy of rt breast (postop mri rt birads 4 non mass enhancement) lesion 17/8/2026 consistent with resection site changes for remdt (is there need for clips or completion matectomy after that as biopsy is negative)

R L



T N M

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Sonomammography:

Clinical Staging:

Biomarker/Molecular

Pathological Staging:

Mets Workup:

Recommendations:

MDT Outcome & Plan

Continue systemic therapy

for completion mastectomy post ttt

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د خالد عبد العزيز	د محمد تيراوي		

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

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Reporting Date	**05/9/2026 11:20:56 AM**	**Pathology No.**	**2607/26**

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Signature
Prof. Dr. Gamal Abd El-Atty.

غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient's Name:	**Arzaq Ibrahim Ahmed Abdel Jawad**
Date:	**2026-08-22**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Neck Ultrasonography

multiple enlarged matted right supracalvicualr L.Ns seen showing irregular shape largest measuring 3cm with hypoechoic centers

The right and left lobes and isthmus of thyroid gland are of normal size, homogenous echo pattern, with regular outline.

Normal vascularity at CCDI.

No cystic or solid nodules detected.

The trachea is centralized.

Both carotid sheaths are free.

Conclusions

Right supraclavicualr highly suspicious pathological L.Ns as described for further assessment

Dr.Sahl Mahmoud

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	ارزاق ابراهيم احمد	**Case No.**	**340122**
Sex/Age	**Female 49 Y**	**Patient No.**	**24416**
Registered Date	**04/7/2026 03:44:50 PM**	**Referred By Dr.**	**Out Patient**
Validation Date	**08/7/2026 11:35:06 AM**	**Branch**	**Gustave Roussy**
Reporting Date	**03/10/2026 09:21:15 AM**	**Pathology No.**	**SR-778/26**

Pathology report
(slide revision)

Clinical Data: Right breast cancer, conservative surgery and Axillary Lymph node dissection were done outside GRI-e

Nature of Specimen: Slide revision

Gross Examination:
-Eight FFPE blocks labelled (202296 and 200595-26) and 17 slides were received for slide revision

Microscopic examination:
- **Histologic type:** Invasive breast carcinoma, NST with medullary like features
- **Histologic grade: 3 (**differentiation 3, pleomorphism 3, mitosis 2)
- **DCIS:** Absent
- **Microcalcification:** Absent
- **Angioinvasion:** Present
- **Perineural invasion:** Absent
- **Resection margins: Refer to previous report**
- **Tumor size: 1x0.5 cm (as described in previous report)**
- **Axillary Lymph nodes: Positive for tumor deposits in 10/15 Lns with extranodal extension**
- **Stage: pT1b N3**

Review of IHC slides
* IPT (All controls showed appropriate reactivity):
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67%: 25%.

Conclusion:
Right breast cancer, Brest conservative surgery and axillary lymph node dissection (slide revision)
- Invasive breast carcinoma, NST with medullary-like features
- Resection margins: Free (as described in previous report)
- Axillary Lymph nodes: Positive for tumor deposits in 10/15 LNs with extranodal extension

CAP ACCREDITED
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IPT (slide revision)
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67%: 25%.

***Scoring system: "ER, PR"*
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
*** Scoring system: "Her2"*
Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Prof. Dr. Maha Guimei